

**Laboratory Investigation Report**

Patient Name	: Hilal Ahmad	Centre	: 1534 - Arctern Healthcare Noida
Age/Gender	: 38 Y 0 M 0 D / M	OP/IP No	: OP/127669
Max ID/Mobile	: 542493/8752005200	Collection Date/Time	: 11/Feb/2022 04:15PM
Lab ID	: 1266022200213	Receiving Date	: 12/Feb/2022
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 15/Feb/2022
Passport No.	:		

Serology Special**Free Testosterone, Serum***

ELISA

Date	11/Feb/2022 04:15PM	Unit	Bio Ref Interval
Free Testosterone	5.01	pg/mL	7.0 - 22.70

Kindly correlate with clinical findings

***** End Of Report *******Dr. Bansidhar Tarai, M.D.**Associate Director
Microbiology & Molecular Diagnostics**Dr. Poornima Sen, M.D.**
Consultant - Microbiology**Dr. Madhuri Somani, M.D., DNB**
Consultant - Microbiology

Page 1 of 11

SIN No: B2B1504198, Test Performed at : 910 - Max Hospital - Saket M S S H, Press Enclave Road, Mandir Marg, Saket, New Delhi, Delhi 110017

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BLOOD CENTRE**Blood Grouping and RH Factor*, EDTA**

Date	11/Feb/2022 04:15PM	Unit	Bio Ref Interval
Blood Group	B POSITIVE		
Haemagglutination			

Kindly correlate with clinical findings

***** End Of Report *****

Dr. Reeta Rai
Senior Consultant & Head
Blood Bank, PPG



Page 2 of 11

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Clinical Biochemistry

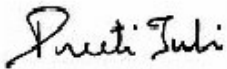
Blood Sugar (Random), Fluoride Plasma

Date	11/Feb/2022 04:15PM	Unit	Bio Ref Interval
Random Glucose UV-Hexokinase	89.4	mg/dl	74 - 140

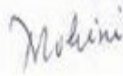
Interpretation A random plasma glucose of > 200 mg/dl with symptoms of diabetes is diagnostic. A confirmatory fasting plasma Glucose (FPG) test or OGTT should be completed on a different day if the clinical condition of the patient permits.

Kindly correlate with clinical findings

*** End Of Report ***



Dr. Preeti Tuli, M.D.
Sr. Consultant - Lab Medicine



Dr. Mohini Bhargava, MD
Principal consultant (Biochemistry)



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Hematology

CBC (Complete Blood Count), Whole Blood EDTA

Date	11/Feb/2022 04:15PM	Unit	Bio Ref Interval
Haemoglobin	12.7	g/dl	13.0 - 17.0
Modified cyanmethemoglobin			
Packed Cell, Volume	38.4	%	40-50
Calculated			
Total Leucocyte Count (TLC)	4.6	10~9/L	4.0-10.0
Electrical Impedance			
RBC Count	4.44	10~12/L	4.5-5.5
Electrical Impedance			
MCV	86.3	fL	83-101
Electrical Impedance			
MCH	28.7	pg	27-32
Calculated			
MCHC	33.2	g/dl	31.5-34.5
Calculated			
Platelet Count	151	10~9/L	150-410
Electrical Impedance			
MPV	12.3	fl	7.8-11.2
Calculated			
RDW	14.3	%	11.5-14.5
Calculated			

Differential Cell Count

VCS / Light Microscopy

Neutrophils	66.2	%	40-80
Lymphocytes	24.9	%	20-40
Monocytes	6.0	%	2-10
Eosinophils	2.2	%	1-6
Basophils	0.7	%	0-2

Absolute Leukocyte Count

Calculated from TLC & DLC

Absolute Neutrophil Count	3.05	10~9/L	2.0-7.0
Absolute Lymphocyte Count	1.2	10~9/L	1.0-3.0
Absolute Monocyte Count	0.28	10~9/L	0.2-1.0



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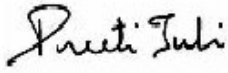
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Hematology

Absolute Eosinophil Count	0.1	10~9/L	0.02-0.5
Absolute Basophil Count	0.03	10~9/L	0.02-0.1

Kindly correlate with clinical findings

*** End Of Report ***



Dr. Preeti Tuli, M.D.
Principal Consultant & Quality Manager
Pathology.



Page 5 of 11

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Immunoassay

Prolactin, Serum

Date	11/Feb/2022 04:15PM	Unit	Bio Ref Interval
Prolactin CLIA	4.85	ng/mL	2.64-13.13

Ref Range

Males :	2.64 - 13.13
Females :	
Premenopausal (<50 years of age):	3.34 - 26.74
Postmenopausal (>50 years of age):	2.74 - 19.64

Interpretation

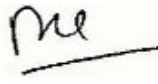
Increased in prolactin-secreting pituitary tumors, amenorrhea and/or galactorrhea, Chiari-Frommel and Argonz Del Cstillo syndromes, various types of hypothalamic-pituitary disease (e.g. sarcoidosis, granulomatous diseases, craniopharyngioma, metastatic cancer, empty sella syndrome), primary hypothyroidism, anorexia nervosa, polycystic ovary syndrome, renal failure, insulin-induced hypoglycemia, chest wall injury, adrenal insufficiency, and pituitary stalk section surgery
Decreased in pituitary apoplexy (Sheehan's Syndrome)

Kindly correlate with clinical findings

*** End Of Report ***



Dr. Poonam. S. Das, M.D.
Principal Director-
Max Lab & Blood Bank Services



Dr. Dilip Kumar M.D.
Associate Director &
Manager Quality



Dr. Nitin Dayal, M.D.
Principal Consultant & Head,
Haematopathology



Page 6 of 11

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Serology			
Test Name	Result	Unit	Bio Ref Interval



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Test Name	Serology Result	Unit	Bio Ref Interval
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HIV Test (I and II),Serum

CLIA

HIV (I and II) Negative

Interpretation

- 1) A Negative result implies that no Anti HIV-1 or HIV-2 antibodies have been detected in the sample by this method. This means that either the patient has not been exposed to HIV-1 or HIV-2 infection or the sample has been taken during the "WINDOW PERIOD" (before the development of detectable levels of antibodies).
- 2) Positive result suggests the possibility of HIV-1 or HIV-2 infection.

Advise:

To rule out false positivity, false negativity and window period, kindly perform "Confirmatory Tests" like HIV I RNA (Qualitative) Real Time PCR.



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Serology

Test Name	Result	Unit	Bio Ref Interval
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Hepatitis B Surface Antigen, Serum

CLIA

HBsAg	Negative		
HBsAg Test Value	0.42		0 - 0.90

Ref. Range

Negative	< 0.90
Borderline	0.90 - 5.0
Positive	> 5.0

Interpretation

- This test is used to detect hepatitis B surface antigen (HBsAg) in serum sample based on VITROS immunometric immunoassay technique and aid in the laboratory diagnosis of HBV infection.
- Viral hepatitis is a major public health problem with an estimated 257 million persistent carriers of hepatitis B virus (HBV) worldwide. Infection with HBV results in a wide spectrum of acute and chronic liver diseases that may lead to cirrhosis and hepatocellular carcinoma.
- Transmission of HBV occurs by percutaneous exposure to blood products, needle stick injury, sexual contact and perinatally from HBV-infected mothers to baby.
- Hepatitis B surface antigen (HBsAg), derived from the viral envelope, is the first antigen to appear following infection.
- Positive results should be correlated with other potential laboratory abnormalities and clinical picture.
- A negative test result does not exclude the possibility of exposure to or infection with hepatitis B virus.
- Levels of HBsAg may be undetectable both in early infection and late after infection.
- In rare cases HBsAg tests do not detect certain HBV mutant strains.
- HBs Ag disappears with recovery from clinical disease in most patients, however, it persists for years in carriers.



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VDRL/RPR, Serum

RPR(Syphilis) Non Reactive
Slide Flocculation

Comment

Interpretation

1. It is a screening test for syphilis which is useful for following the progression of disease and response to therapy. Rising titers are of immense value in confirming the diagnosis.
2. Biological false positive reactions exhibit low titers and are seen in conditions like Viral fevers, Mycoplasma infection, Chlamydia infection, Malaria, Immunizations, Pregnancy, Autoimmune disorders & past history of Treponemal infection.
3. It is advisable to confirm the diagnosis by tests such as TPHA & FTA-ABS.
4. A definitive clinical diagnosis should not be made by result of a single test only, but should be made by taking clinical history and other laboratory findings in to account.

Kindly correlate with clinical findings

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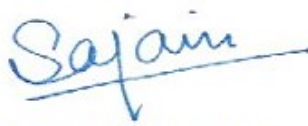
Dr. Ranjana Chhabra, M.D.
Senior Consultant Microbiology



Dr. Ranjana Chhabra, M.D.
Senior Consultant Microbiology



Dr. Suchitra Jain, M.D.
Senior Consultant Microbiology



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Test Name	Serology Result	Unit	Bio Ref Interval
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Dr. Saloni Sehgal (MBBS, MD)
Principal Consultant &
Head Microbiology & Infection Control



Dr. Neera Kaushik
Senior Microbiologist

Results to follow:

Karyotyping-Peripheral Blood Cells : 01/Mar/2022 07:00 PM



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Max Lab, Max Hospital, Gurgaon: Opposite HUDA City Centre Metro Station, B-Block, Sushant Lok-1, Gurgaon-122001,
Phone: +91-124-6623 000 | (CIN No.: U85100DL2021PLC381826)

Helpline No. 7982 100 200 | www.maxlab.co.in | feedback@maxlab.co.in

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HILAL AHMAD ML01777832

PID NO: P542100890866

Age: 38.0 Year(s) Sex: Male



Reference: Dr.DR SELF

Sample Collected At:

Max Healthcare Institute Ltd
1, Press Enclave Road, Saket, New Delhi-
110017,gst No-07aadcm0815b1z4Zone:
Patparganj

Processing Location:- Metropolis
Healthcare Ltd, Unit No. 409- 416, 4th
Floor, Commercial Building-1, Kohinoor
Mall, Mumbai-70

VID: 542131500643522

Registered On:

12/02/2022 01:49 PM

Collected On:

13/02/2022 3:09AM

Reported On:

21/02/2022 04:20 PM

Y-CHROMOSOME MICRODELETION ANALYSIS

Specimen

EDTA Whole Blood

Method

Multiplex PCR for Microdeletion in STS markers of AZF region.

Y-Chromosome Microdeletion PCR detects the following STS markers in the AZFa, b, and c region of Y-chromosome.

- AZFa: sY81, sY84s, sY86, sY182
- AZFb: sY121, sY124, sY127, sY128, sY130, sY134
- AZFc: sY153, sY157, sY254, sY255, sY145, sY152

Clinical Indication

Infertility

Results

Y-Chromosome Microdeletion not detected. None of the listed markers spanning the AZFa, AZFb and AZFc regions are deleted.

Interpretation

This study does not exclude other possible alterations (for example point mutations) in the Y-chromosome or other chromosomal abnormalities. Karyotype analysis, if not performed already, is advised.

Clinical implications of Y-chromosome microdeletions:

- Microdeletions of the azoospermia factor (AZF) regions on the q arm of the Y chromosome are present in 5–10 percent of males with non-obstructive azoospermia or severe oligospermia. They are not found in normospermic men, proving there is clearly a cause-and-effect relationship between Y deletions and spermatogenic failure.
- The highest frequency of Y deletions is found in azoospermic men (8-12%), followed by
- oligozoospermic (3-7%) men. Deletions are extremely rare with a sperm concentration >5 million of spermatozoa/mL (approximately 0.7%).
- AZFc deletions are most common (approximately 65-70%), followed by deletions of the AZFb and AZFb+c or AZFa+b+c regions (25-30%). AZFa region deletions are extremely rare (5%).
- Deletions of AZFa are rare but can be associated with a very severe phenotype and complete absence of spermatogenesis or azoospermia.
- AZFb deletions are associated with zero probability of finding mature spermatozoa due to developmental arrest of germ cells.
- Complete, bilateral Sertoli-cell-only (SCO) is the histological feature of AZFa deletions and 50% of AZFb deletions.
- Patients with AZFc deletions have a slightly milder phenotype with mixed atrophy and residual spermatogenesis in about 50% of patients, although SCO is present in the majority of these patients.
- TESE is possible in patients with AZFc deletions with a probability of recovering sperm of about 50%, but no sperm retrieval has been so far reported in patients with complete AZFa or AZFb deletions.
- Testing for microdeletions is not necessary in men with obstructive azoospermia when ICSI is used because spermatogenesis should be normal. If a man with microdeletion wishes to proceed with ICSI, they should be advised that microdeletions will be passed to sons, but not to daughters.
- Screening in male relative is recommended in case of Partial AZFa or AZFb and AZFc deletion. Complete AZFa, AZFb,

Dr. Kirti Chadha
Chief Scientific Officer,
Senior Oncopathologist



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AZFbc or AZFabc deletion are generally incompatible with sperm production thus family screening is not indicated. Genetic counseling is recommended.

Indications for Testing:

The test is useful in males diagnosed with non-obstructive azoospermia or moderate to severe oligospermia, idiopathic infertility and those undergoing testicular sperm extraction (TESE) or prior to ICSI.

Note:

This test has been developed and its performance is validated at Metropolis Healthcare Ltd.

Limitation of the Assay:

Presence of PCR inhibitors in the sample may prevent DNA amplification. Paradoxical results may arise due selection of inappropriate specimens and contamination during specimen collection.

References:

- Krausz et al.; 2014, Andrology; 2(1): 5–19.
- Abilash et al., 2010, International Journal of Genetics and Molecular Biology, Vol. 2(7), pp. 147-157
- Simoni et al., 2004, International Journal of Andrology, 27:240–249
- Sun et al., 2012, The Journal of International Medical Research, Vol. 40, No. 1, pp. 237-248

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